

I M Smrti — Topic Bank Playbook

65 ready-to-shoot topics. For each: what it's about, the hook, exactly what to say, and how to end it. Open this, pick a number, shoot.

Updated: 8 September 2026

How to use this

1. Pick any topic below — story bank first (see `CONTENT-STRATEGY.md` §9), otherwise work down the list in order.
2. Read the **Say** block out loud once before filming. It's talking points, not a script — say it in your own words.
3. Every reel: **on-screen disclaimer at 0:00**, small, top of frame, 2 seconds: Not a doctor. Health information, not medical advice.
4. Every post ends on the **handoff** given below — it turns your story into their action.
5. Full voice, format, CTA and posting rules live in `CONTENT-STRATEGY.md`. This document is the shortcut so you never have to open that one mid-shoot.

The compliance line, once, so you don't have to think about it 65 times: Say what the report/test **shows**, then what to **ask your doctor**. Never what to *do*. No dosage, no "you need X supplement," no telling anyone to stop a medicine, no brand endorsement.

PILLAR 1 — DECODE THE REPORT

Your unfair advantage: 66 tests already explained in plain language inside the app.

1. 1-0-1 का मतलब — your prescription is written in code

Format: Carousel or 30s reel **Hook:** "Your prescription is written in code. Here's the key." **Say:** Three positions on a strip = morning–afternoon–night. 1-0-1 = one tablet morning, one at night, nothing afternoon. 0-0-1 = night only. 1-1-1 = three times a day. ½-0-½ = half tablet morning and night. **End:** "Save this before your next doctor visit. Send it to whoever picks up your family's medicines."

2. OD, BD, TDS, HS — what your doctor's shorthand actually means

Format: Carousel **Hook:** "OD. BD. TDS. Your doctor writes in Latin and nobody tells you." **Say:** OD = once a day. BD = twice a day (every 12 hours, not "twice whenever"). TDS/TID = three times a day. QID/QDS = four times a day. HS = at bedtime. **End:** "If you've ever nodded at the pharmacist without understanding the label — save this."

3. SOS on a prescription doesn't mean emergency

Format: Reel, 20–25s **Hook:** "SOS on your prescription does NOT mean what you think." **Say:** SOS means "only if needed" — not emergency, not daily. A painkiller marked SOS was never meant to be taken every day. Contrast with STAT, which means immediately. **End:** "Check the medicine cabinet at home right now. Is anything marked SOS being taken like a daily tablet?"

4. "Khaane ke baad" = 30 minutes after, not with the last bite

Format: Reel, 20s **Hook:** "Khaane ke baad doesn't mean what you're doing." **Say:** "After food" means roughly 30 minutes after you finish eating — not swallowed with your last bite of roti. AC (before food) vs PC (after food). Some medicines genuinely don't work right if the timing is off. **End:** "Ask your doctor: kitni baar, aur khaane se pehle ya baad? Ten seconds that saves a week of guessing."

5. Same medicine, ₹12 and ₹180 — generic vs brand

Format: Carousel **Hook:** "Same medicine. ₹12 and ₹180. Nobody told you." **Say:** The active molecule is identical; the brand name is what costs more. Show one real example (e.g. Paracetamol vs a branded equivalent). Explain that pharmacists can usually substitute generic for brand unless the doctor writes "no substitution." **End:** "Next time, ask your chemist for the generic. Send this to whoever manages the family's medicine budget."

6. Jan Aushadhi — same molecule, up to 70% cheaper

Format: Reel, 30s **Hook:** "There's a government pharmacy chain selling medicines up to 70% cheaper. Most people don't know it exists." **Say:** Jan Aushadhi Kendras sell generic versions of common medicines at a fraction of branded price. Explain how to find one (search "Jan Aushadhi near me"), and that it's the same molecule, government-certified. **End:** "If your parents are on long-term medication, this alone could save thousands a month. Send it to them."

7. Why stopping antibiotics early is worse than never starting

Format: Reel, 30–40s **Hook:** "Feeling better and stopping your antibiotic early is worse than not taking it at all." **Say:** Stopping early lets the strongest bacteria survive and multiply — this is how antibiotic resistance happens, in your body and beyond. The course length exists for a reason even after symptoms disappear. **End:** "Finish the full course your doctor prescribed — even when you feel fine on day 3."

8. That 60,000 IU Vitamin D sachet — why weekly, not daily

Format: Reel, 30s **Hook:** "Vitad3 60K — why is this taken once a WEEK, not once a day?" **Say:** 60,000 IU is a large dose, calibrated for weekly correction of deficiency, not daily maintenance. Taking it daily would be a significant overdose. This is exactly why self-medicating on Vitamin D is risky — the dose has to match the deficiency, decided by a blood test and a doctor. **End:** "Don't guess your own dose. Get tested, then ask your doctor exactly how often."

9. Vitamin D 8.4 when normal starts at 30 — what "deficient" really means

Format: Reel, 45–55s — *(full script already written in CONTENT-SCRIPTS-WEEK-01.md, post 3)* **Hook:** "This report said 8.4. Normal starts at 30." **Say:** That's severe deficiency, not mild. Symptoms: tiredness sleep doesn't fix, body ache, bone/joint pain, weak muscles, low mood. Cite Metropolis data: 2.2 million tests, 46.5% deficient, 26% insufficient — roughly 7 in 10 inadequate. Why India specifically: indoor lifestyles, covering up, pollution blocking UV, low dietary intake. **End:** "Get tested. One blood test might explain years of unexplained tiredness."

10. Haemoglobin below 12 in women: common, not normal

Format: Carousel **Hook:** "57% of Indian women are anaemic. We've started treating it as normal. It isn't." **Say:** Cite NFHS-5: 57% of women 15–49 anaemic, up from 53% in the previous survey — it's getting worse, not better. Normal haemoglobin for women is roughly 12+ g/dL. Symptoms often dismissed as "just being tired": fatigue, breathlessness, pale skin, hair fall. **End:** "If you've been tired for months and called it normal — it might not be. Ask for a CBC test."

11. HbA1c — three months of truth in one number

Format: Reel, 40s **Hook:** "One number on your report tells three months of truth. Most people have never heard of it." **Say:** Fasting sugar shows one moment; HbA1c reflects average blood sugar over roughly the last 3 months, because it measures sugar attached to red blood cells. That's why a normal fasting reading can hide a real problem. **End:** "If diabetes runs in your family, ask specifically for HbA1c — not just fasting sugar."

12. Fasting sugar normal but HbA1c high? Here's what's happening

Format: Reel, 30s **Hook:** "Fasting sugar: normal. HbA1c: high. How is that possible?" **Say:** Fasting sugar is a snapshot taken once; it can look fine if you happened to test on a good day. HbA1c averages three months, so it catches spikes that a single fasting test misses — especially after meals. **End:** "One test isn't the full picture. Ask your doctor which combination makes sense for you."

13. TSH high = thyroid slow = tired, cold, gaining weight

Format: Carousel **Hook:** "Tired all the time, always cold, weight not moving? One number might explain it." **Say:** TSH (thyroid stimulating hormone) rising usually means the thyroid is underactive (hypothyroid). Symptoms: fatigue, cold intolerance, weight gain, hair thinning, low mood. Extremely common and very treatable once diagnosed. **End:** "If this list sounds like you, ask for a simple TSH test — it's inexpensive and answers a lot."

14. SGPT raised and you don't drink? Probably fatty liver

Format: Reel, 40s **Hook:** "SGPT is raised and you don't drink alcohol. So what's going on?" **Say:** SGPT/SGOT are liver enzymes; raised values without alcohol usually point to fatty liver (NAFLD). Cite the number: roughly 1 in 3 Indian adults has some degree of fatty liver. Linked to diet, obesity, sugar — not just alcohol. **End:** "Fatty liver is very common and, caught early, very manageable. Show this reading to your doctor."

15. Why two labs give different "normal" for the same test

Format: Reel, 25s **Hook:** "Same blood. Two labs. Two different 'normal' ranges. Why?" **Say:** Labs calibrate reference ranges to their own equipment, method, and reference population — so ranges legitimately vary slightly between labs. The number that matters is where you fall relative to *that specific report's* range, not a number you remember from before. **End:** "Always read the range printed on your own report — not a number from memory."

16. What actually needs 12-hour fasting

Format: Carousel **Hook:** "You don't need to starve for every blood test. Here's what actually needs fasting." **Say:** Fasting sugar and lipid profile (cholesterol) genuinely need 8–12 hours fasting for accurate results. Most other tests (CBC, thyroid, liver, kidney) don't require fasting at all — many people fast unnecessarily out of habit. **End:** "Ask the lab directly — don't assume every test needs an empty stomach."

17. Platelet count in dengue — when to actually worry

Format: Reel, 30s — (*seasonal: monsoon/post-monsoon*) **Hook:** "Platelet count dropping in dengue — when is it actually dangerous?" **Say:** Normal platelet range is roughly 1.5–4.5 lakh/ μ L. In dengue, counts are watched over serial reports, not one reading — the trend and bleeding symptoms matter more than a single number. This is a "monitor with a doctor" situation, not a home-management one. **End:** "In dengue season, don't just track one report — track the trend, with your doctor."

18. B12 deficiency: tingling hands, poor memory, vegetarian diet

Format: Reel, 30s **Hook:** "Tingling in your hands and feet? It might not be what you think." **Say:** B12 comes almost entirely from animal products, so it's common in vegetarian/vegan diets — very relevant in India. Symptoms: tingling/numbness in hands and feet, fatigue, poor concentration, memory issues. **End:** "If you're vegetarian and these symptoms sound familiar, ask for a B12 test."

19. Creatinine — the kidney number ignored until it's late

Format: Reel, 30s **Hook:** "Creatinine is the one number people ignore — until it's too late." **Say:** Creatinine reflects kidney filtering function; it often stays "normal-looking" until kidney function has already dropped significantly, because kidneys have a lot of reserve capacity. That's why it's a number worth tracking over time, not just once. **End:** "If diabetes or high BP runs in your family, ask for creatinine at your next checkup — even if you feel fine."

20. Uric acid high? That big-toe pain has a name

Format: Reel, 25s **Hook:** "That sudden, sharp pain in your big toe at 3am — it has a name." **Say:** High uric acid can crystallise in joints, most classically the big toe, causing gout — sudden, severe pain and swelling. Linked to diet (red meat, alcohol, sugary drinks) and can be picked up on a simple blood test before an attack happens. **End:** "If gout runs in your family, ask about uric acid at your next blood test."

21. HDL going UP is good news — here's why

Format: Reel, 20s **Hook:** "Most numbers on your report — you want them down. This one you want UP." **Say:** HDL is "good cholesterol" — it helps clear other cholesterol from your bloodstream, so a higher HDL is protective, unlike LDL and triglycerides. Regular exercise is one of the most reliable ways to raise it. **End:** "Next time you see HDL on a report, remember: higher is the good direction here."

22. ESR/CRP raised = "inflammation somewhere" — what that means

Format: Reel, 25s **Hook:** "Your report says 'inflammation markers raised.' Raised because of what?" **Say:** ESR and CRP are non-specific — they say the body is fighting something (infection, injury, chronic condition) but not what or where. On their own they're a signal to investigate further, not a diagnosis. **End:** "If these are raised on your report, that's the start of a conversation with your doctor, not the end of one."

23. 4 questions to ask when a doctor prescribes something new

Format: Carousel **Hook:** "4 questions to ask before you leave the room with a new prescription." **Say:** 1) What is this for? 2) How and when do I take it? 3) How long is the course? 4) What side effects should worry me? Ten seconds each, and you walk out actually understanding your treatment. **End:** "Save this. Read it before your next appointment."

PILLAR 2 — TASTY ≠ UNHEALTHY

Your enemy: boiled-food advice copied from a culture that doesn't cook like us. Stay in your lane — habits, timing, portions, swaps. No metabolism claims, no studies you haven't read.

24. Your dal-chawal is fine. Your portion isn't

Format: Reel, 30s **Hook:** "Your dal-chawal isn't the problem. Nobody told you about the portion." **Say:** Home-cooked Indian staples aren't inherently unhealthy — the issue is usually portion size and what sits alongside them (fried sides, sugary drinks, seconds out of habit rather than hunger). **End:** "You don't need to change what you cook. Notice what's actually on the plate."

25. Ghee isn't the villain. The quantity is

Format: Reel, 30s — (full script in `CONTENT-SCRIPTS-WEEK-01.md`, part of post 2's theme — this is a standalone extension) **Hook:** "Ghee isn't the villain. The quantity is." **Say:** Ghee in moderate amounts is a normal part of Indian cooking; the problem is usually volume — a generous daily habit adds up in calories regardless of how "natural" the fat is. **End:** "You don't have to give it up. Just notice how much is going in."

26. Boiled food is not health food — the biggest lie on Indian health pages

Format: Reel, 45s — *(full script already written: CONTENT-SCRIPTS-WEEK-01.md , post 2)* **Hook:** "Boiled food is not health food." **Say:** Most Indian health advice is copied from Western restriction culture. The real problem was never spice or ghee — it's the packet, the portion, the 4pm biscuit. You removed junk from your own diet, not flavour. **End:** "Cook your food. Eat your food. Just stop eating the packet."

27. 5 Indian swaps that change nothing about taste

Format: Carousel **Hook:** "5 swaps that change your health, not your taste." **Say:** Concrete, specific swaps you've actually tested — e.g. cooking method tweaks, a smarter snack replacing the 4pm biscuit, a portion-neutral way to keep a dish's flavour with less oil. *(Fill in your own 5 — this is the one topic where your personal kitchen experiments are the content.)* **End:** "Try one this week. Tell me which one in the comments."

28. Why we can cook healthy better than any Western diet plan

Format: Reel, 30s **Hook:** "Every diet plan online is written for a country that doesn't cook like we do." **Say:** Indian cooking already has the ingredients — turmeric, ginger, garlic, whole spices, lentils, vegetables cooked fresh daily. The "problem" framing (avoid Indian food, eat Western-style salads) ignores that home-cooked Indian food, done with attention to portion and oil, is a strong foundation most diet culture doesn't credit. **End:** "You don't need a diet plan from another country. You need attention on the one you already have."

29. The 4pm chai-biscuit does more damage than your dinner

Format: Reel, 25s **Hook:** "The 4pm chai-biscuit does more damage than your entire dinner." **Say:** It's easy to dismiss a small daily habit as harmless, but a repeated snack — sugar in the tea, 2–3 biscuits, every single day — adds up to more sugar and refined flour over a week than most people realise, precisely because it doesn't feel like "a meal." **End:** "Track your actual 4pm habit for three days. You might be surprised."

30. Spices that actually do something (and the ones that are just marketing)

Format: Carousel **Hook:** "Which Indian spices actually do something — and which are just marketing?" **Say:** Turmeric (curcumin, anti-inflammatory properties studied), ginger (digestion, nausea), fenugreek/methi (blood sugar). Be honest about the limits: these are supportive, not medicine, and no spice replaces treatment. Avoid overclaiming — cite generally accepted culinary-medicinal use rather than specific study numbers. **End:** "None of these replace your medicine. They're a bonus, not a substitute."

31. Same sabzi, half the oil, no taste difference — the technique

Format: Reel, 30s **Hook:** "Same sabzi. Half the oil. Same taste. Here's the technique." **Say:** A real, specific cooking technique you use — e.g. tempering spices in a smaller amount of hot oil first for flavour, then finishing without extra oil; or covering the pan to cook vegetables in their own moisture instead of extra oil. **End:** "Try it in your next sabzi and taste the difference — or lack of one."

32. Roti vs rice — the question is wrong, here's the real one

Format: Reel, 25s **Hook:** "Roti vs rice — everyone's asking the wrong question." **Say:** Both are fine staples in a balanced meal; the real variable is portion and what else is on the plate (protein, vegetables), not which grain wins some imaginary contest. This debate distracts from the things that actually matter. **End:** "Stop debating roti vs rice. Start looking at your full plate."

33. What's actually inside packaged "healthy" snacks

Format: Carousel **Hook:** "That 'healthy' packaged snack — let's actually read the label." **Say:** Take one real product's label and walk through it: sugar content, "multigrain" vs actual whole grain content, hidden oils, serving size tricks (small stated serving to make numbers look better). Teach the skill of reading a label, not just this one product. **End:** "Next time you pick up a packet, flip it over before you decide."

34. Protein in a vegetarian Indian diet — where it actually comes from

Format: Carousel **Hook:** "You're vegetarian. Where does your protein actually come from?" **Say:** Dal, rajma, chole, paneer, curd, milk, soya, sprouts, peanuts — a normal Indian vegetarian diet has real protein sources; the myth is that vegetarians can't get enough. The gap is usually not knowing to combine or prioritise these, not the absence of options. **End:** "You probably eat more protein sources than you give yourself credit for. Count your plate this week."

35. Why Indians get diabetes at a lower BMI than Europeans

Format: Reel, 35s **Hook:** "Indians get diabetes at a BMI that would be called 'normal' in Europe. Why?" **Say:** Indians tend to carry more visceral (internal, abdominal) fat at a given BMI than Europeans — sometimes called the "thin-fat Indian" phenotype — meaning standard BMI cutoffs underestimate risk for our population. This is part of why ICMR-INDIAB found abdominal obesity in 39.5% of Indians even where BMI looks fine. **End:** "BMI alone doesn't tell the whole story for us. Waist measurement matters too — ask your doctor."

36. Salt: the 5g limit, and how one spoon of achaar blows past it

Format: Reel, 25s **Hook:** "5 grams of salt a day — that's the WHO limit. One spoon of achaar can wreck it." **Say:** WHO recommends under 5g salt/day; pickle (achaar), papad, and packaged snacks are salt-dense in ways people don't account for, on top of the salt already used in cooking. It's not about giving these up — it's about knowing they count. **End:** "You don't have to stop the achaar. Just know it's not 'free.'"

PILLAR 3 — SUBTRACT FIRST

Your method, lived not borrowed: remove junk before adding supplements.

37. Stop adding supplements. Remove three things instead

Format: Reel, 30s **Hook:** "Stop adding another supplement. Remove three things instead." **Say:** The instinct is always to add — a new powder, a new pill. The bigger lever for most people is removing sugar, alcohol, or ultra-processed food first, before spending money on anything new. **End:** "Before your next supplement purchase, ask: what could I remove instead?"

38. I removed sugar, alcohol and fast food. I added nothing. Here's what happened

Format: Reel, 40s — (*your own story, lived experience* — see *CONTENT-STRATEGY.md* §5) **Hook:** "I removed sugar, alcohol and fast food. I added nothing. Here's what happened." **Say:** Three months in the gym, and the biggest change wasn't a new supplement — it was subtraction. No sugar, no alcohol, no fast food. Everything else stayed the same. Talk honestly about what changed (energy, focus, control) without medical claims about outcomes like weight or blood markers unless you've actually tested them. **End:** "What's one thing you could remove this week, instead of adding something new?"

39. Most Indians need 2–3 supplements, not 8

Format: Reel, 30s **Hook:** "You're probably taking 6 supplements. You probably need 2." **Say:** Common over-supplementing pattern: multivitamin + protein + omega-3 + biotin + ACV + more, often without testing. Many of these overlap or aren't needed unless a blood test shows a specific deficiency. **End:** "Get tested before you buy the next one. You might already be covered."

40. Get tested before you supplement — the ₹800 test that saves ₹30,000

Format: Reel, 25s **Hook:** "One ₹800 test could save you ₹30,000 a year on supplements you don't need." **Say:** A basic panel (Vitamin D, B12, iron/ferritin as relevant) costs a fraction of a year of guessing with supplements. Testing first means you spend on what you're actually short on — not what an ad told you to buy. **End:** "Before you buy the next bottle, get the test instead."

41. Protein powder: who needs it, who's burning ₹3,000/month

Format: Reel, 30s **Hook:** "Protein powder — who actually needs it, and who's just burning ₹3,000 a month?" **Say:** If your diet already includes dal, paneer, curd, eggs, or meat in reasonable quantity, many people are closer to their protein needs than gym marketing suggests. Powder is a convenience for a gap, not a requirement for everyone lifting weights. **End:** "Before you buy another tub, actually count what you're eating for a few days."

42. Multivitamin daily — helpful or expensive urine?

Format: Reel, 20s **Hook:** "Is your daily multivitamin doing anything — or is it just expensive urine?" **Say:** Water-soluble vitamins taken in excess of what the body needs are largely excreted, not stored — hence the joke. A multivitamin taken without any known deficiency may be doing very little beyond a placebo habit. Testing tells you if it's actually needed. **End:** "If you've never been tested, that daily multivitamin is a guess, not a plan."

43. Calcium without Vitamin D is money down the drain

Format: Reel, 25s **Hook:** "Taking calcium without Vitamin D? You might be wasting your money." **Say:** Vitamin D is what allows the body to actually absorb calcium — without adequate D, extra calcium isn't used as effectively. This is why the two are often prescribed together, not separately. **End:** "If you're on a calcium supplement alone, ask your doctor whether your Vitamin D has been checked too."

44. "Immunity booster" is a marketing word, not a medical one

Format: Reel, 25s **Hook:** "'Immunity booster' — that's a marketing word. Not a medical one." **Say:** The immune system doesn't work like a dial you turn up with a syrup or a powder — it's a complex, balanced system. What genuinely supports it: sleep, balanced nutrition, movement, stress management, and treating actual deficiencies. No product "boosts" it in the way ads imply. **End:** "Next time you see 'immunity booster' on packaging, read it as a marketing word, not a medical claim."

45. The one-week subtraction experiment

Format: Reel or Carousel, 30s **Hook:** "Try this for one week: don't add anything. Just remove one thing." **Say:** Pick one thing to remove for 7 days — sugar in tea, the evening biscuit, one alcoholic drink a week, whatever's honest for you. Track how you feel, without expecting a transformation. This is about testing the "subtract first" philosophy for yourself. **End:** "Pick your one thing. Tell me in the comments what you're removing this week."

PILLAR 4 — INPUTS, NOT OUTCOMES

Your method: fix the input, not the outcome. Lived, not theoretical.

46. I wake at 4am without an alarm — I never fixed my mornings, I fixed my nights

Format: Reel, 40s — (your own story) **Hook:** "I wake at 4am without an alarm. I never tried to fix my mornings." **Say:** The instinct is to force a wake-up time with an alarm, fighting your own body. Instead, fix the sleep (input) — a consistent, earlier bedtime — and the wake-up (output) follows naturally. This is the whole "fix inputs, not outcomes" philosophy in one lived example. **End:** "If you're fighting your alarm every morning, you might be solving the wrong end of the problem."

47. Fix the input, not the outcome — the whole philosophy in 40 seconds

Format: Reel, 40s **Hook:** "One idea changed more about my health than any diet or workout plan." **Say:** Most people try to force an outcome directly — force yourself awake, force yourself to eat less. It rarely holds. Fixing the input that produces the outcome holds much better — sleep time instead of wake time, removing a food instead of counting calories, an eating window instead of willpower at every meal. **End:** "Next time you're trying to force an outcome, ask: what's the input actually driving this?"

48. Eating windows beat calorie counting for most people

Format: Reel, 30s **Hook:** "I don't count calories. I just know my eating window." **Say:** Instead of tracking every number, define a consistent window for when you eat — e.g. lunch always 1:30–2:15, dinner always around the same time, nothing outside it. This removes hundreds of small daily decisions rather than requiring constant tracking. **End:** "Try defining just your eating window this week — nothing else."

49. No munching between meals — the smallest change with the biggest result

Format: Reel, 20s **Hook:** "No munching between meals. The smallest rule, the biggest result." **Say:** Constant small snacking keeps insulin and digestion working continuously with no real break — a defined gap between meals lets the body actually rest between them. This is one habit, not a diet plan, and it's simple to start. **End:** "Try going meal to meal with zero munching for just three days."

50. A 10-minute walk after dinner: the cheapest sugar control that exists

Format: Reel, 20s **Hook:** "The cheapest blood sugar control that exists costs ₹0 and takes 10 minutes." **Say:** A short walk after eating helps the body use up the glucose from the meal instead of it sitting in the bloodstream — a simple, free, well-supported habit, especially relevant given how common elevated blood sugar is in India. **End:** "Tonight, after dinner — just 10 minutes. That's the whole ask."

51. Standing up every 45 minutes undoes most of the damage of sitting

Format: Reel, 20s **Hook:** "You don't need a standing desk. You need to stand up every 45 minutes." **Say:** Long, unbroken sitting is what causes most of the harm associated with a sedentary lifestyle — a brief break to stand and move every 45 minutes or so meaningfully changes that, without needing new equipment or a big lifestyle change. **End:** "Set a reminder right now for every 45 minutes at work tomorrow."

52. Sleeping at 2am and "getting 8 hours" is not the same thing

Format: Reel, 25s **Hook:** "Sleeping from 2am to 10am is NOT the same as sleeping from 10pm to 6am." **Say:** Sleep quality and hormone regulation are tied to your body's natural circadian rhythm, not just total hours — late, shifted sleep doesn't give the same recovery as earlier, consistent sleep, even at the same duration. **End:** "Same 8 hours, different clock — the timing matters more than people think."

53. Why saying NO fixes more than saying YES to something new

Format: Reel, 25s **Hook:** "Saying no fixed more of my health than any new habit I added." **Say:** Adding new habits (a new workout, a new supplement, a new diet) competes for willpower and time. Removing something (sugar, a late-night habit, a snack) is often a one-time decision that keeps paying off, rather than a daily effort. **End:** "What's one 'yes' in your routine that might actually need to become a 'no'?"

PILLAR 5 — FOR YOUR PARENTS (60+)

Different audience, different language: yoga, walking, Ayurveda — not gym, not macros.

54. My father is 60, 90kg, and just started walking. Here's what we're tracking

Format: Carousel — *(with your father's permission)* **Hook:** "My father is 60, 90kg, and just started walking. Here's what we're tracking." **Say:** Share the real, ongoing situation honestly — his starting point, why walking (not gym) was the right first step for him, what you're both watching (weight trend, energy, how his joints feel) without inventing medical claims about outcomes. **End:** "If your parent needs a first step, it doesn't have to be the gym. It can be a walk."

55. घुटनों में दर्द? It may not be age — it may be one vitamin

Format: Reel, 30s **Hook:** "घुटनों में दर्द को लोग सिर्फ उम्र समझ लेते हैं। शायद ऐसा नहीं है।" **Say:** Joint pain is often assumed to simply be "old age," but low Vitamin D specifically presents as bone and joint pain — and it's extremely common and correctable. Not every ache is inevitable; some are a treatable deficiency. **End:** "Before accepting knee pain as 'just age,' get a Vitamin D test."

56. Yoga vs gym after 60 — what actually helps

Format: Carousel **Hook:** "Gym or yoga after 60 — what actually helps?" **Say:** For most people over 60, gentle, joint-friendly movement (yoga, walking, stretching) sustainably builds consistency better than high-intensity gym routines, which carry more injury risk without supervision. The goal at this age is consistency and mobility, not intensity. **End:** "It doesn't have to be a gym. Consistency matters more than intensity at this stage."

57. Pranayam and blood pressure: what the evidence says

Format: Reel, 30s **Hook:** "Can pranayam actually help blood pressure? Here's what's understood." **Say:** Controlled breathing practices are associated with the body's relaxation response, which can support blood pressure management as part of a broader routine — but it's a complement to medical treatment, not a replacement for it. Be careful not to overclaim; frame as supportive, not curative. **End:** "If BP runs in your family, this is worth adding — alongside your doctor's plan, not instead of it."

58. Digestion after 60 — why timing matters more than food

Format: Reel, 25s **Hook:** "After 60, digestion often has less to do with WHAT you eat and more with WHEN." **Say:** Digestive efficiency can slow with age; consistent meal timing, smaller portions, and not eating too close to bedtime often help more than dramatic diet changes. Practical, gentle adjustments rather than restriction. **End:** "Try shifting dinner earlier for your parent this week — even by 30 minutes."

59. The medicine list your parent should carry in their wallet

Format: Carousel **Hook:** "One folded piece of paper in your parent's wallet could matter in an emergency."

Say: A simple list: current medicines and doses, known allergies, blood group, emergency contact, any major conditions. In an emergency, this is often the information a stranger or paramedic needs most and least likely to have. **End:** "Make this list for your parent this week. It takes ten minutes."

PILLAR 6 — THE CAREGIVER'S JOB

This is you. This is where the founder story lives.

60. Two hours of waiting. Five minutes with the doctor. Here's how to use them

Format: Reel, 40s — *(ties directly to the founder story)* **Hook:** "Two hours of waiting. Five minutes with the doctor. Here's how to actually use them." **Say:** Prep before you go in: write down the main concern in one sentence, list current medicines, note any changes since the last visit. Walking in with this means the five minutes go to decisions, not to you recalling history from memory under pressure. **End:** "Next appointment, write this down on the way there. It changes how the whole visit goes."

61. Your parents won't tell you they're unwell — what to watch for

Format: Carousel **Hook:** "Your parents won't tell you they're unwell. Here's what to actually watch for." **Say:** Common signs parents underreport: unusual tiredness, skipping normal activities, appetite changes, sleeping more, complaining less than usual (not more) about aches they'd normally mention. Silence is sometimes the signal, not complaints. **End:** "If something feels 'off' but they say they're fine — trust the feeling and ask again."

62. In an emergency, does anyone know your mother's blood group?

Format: Reel, 25s — *(ties to the app's emergency card feature)* **Hook:** "In an emergency, does anyone know your mother's blood group?" **Say:** The person who knows the medical history is often the person who isn't there in an emergency. A simple, accessible record — blood group, allergies, current medicines — removes that gap for anyone who needs to help. **End:** "Find out today, if you don't already know. Write it down somewhere accessible."

63. Managing parents' health from another city — what actually works

Format: Carousel **Hook:** "Managing your parents' health from another city — what actually works." **Say:** A shared system beats relying on memory and anxious phone calls: one shared family channel for updates, a shared calendar for appointments, reminders for daily medicines, someone locally who can be a first point of contact if you can't be there. **End:** "If you manage your parents' health remotely, send this to the sibling who's local — build the system together."

64. Every Indian family has an unpaid Chief Health Officer

Format: Reel, 30s **Hook:** "Every Indian family has an unpaid Chief Health Officer. You already know who it is in yours." **Say:** One person in most families ends up tracking everyone's appointments, medicines, and reports — often without it ever being formally decided, and often without acknowledgment. Naming the role is the first step to making it easier, or sharing it. **End:** "Tag the person in your family who does this. They'll know exactly what you mean."

65. What to carry to a hospital visit (and what you'll forget)

Format: Carousel **Hook:** "What to actually carry to a hospital visit — and the one thing you'll forget." **Say:** ID, insurance/health card, current medicine list or the actual strips, previous relevant reports, a written note of symptoms and timeline. The most commonly forgotten item: the most recent report, because it's assumed the doctor already has it. **End:** "Screenshot this list. Keep it for the next visit."

After you shoot

- Reply to every comment in the first hour — it's a ranked signal, not just courtesy ([CONTENT-DISTRIBUTION.md](#) §2)
- Log new questions people ask into the story bank — that becomes next week's topics
- Track **saves and sends**, not likes ([CONTENT-STRATEGY.md](#) §17)
- Full posting cadence, CTA rotation and the first-30-days sequence: [CONTENT-STRATEGY.md](#) §12–15
- Full compliance rules: [CONTENT-STRATEGY.md](#) §12, "Safety + compliance rule"